

PROK pitch deck

PITCH — PROK — VERDICT

PROPOSED — PENDING ADJUDICATION — no position, no order, no entry today; both benches land on “don’t buy,” and the honest reason is that the stock is roughly fairly priced for a coin flip we cannot measure.

What we believe

ProKidney is a single-product company. The product is rilparencel — a cell therapy made from a patient’s own kidney tissue, re-injected to slow chronic kidney disease in diabetic patients. Everything about the company rides on one late-stage trial (PROACT 1) reading out around mid-2027.

Our view, after both benches argued it:

1. **The headline efficacy number does not mean what the bulls say it means.** The Phase 2 trial (REGEN-007) reported that treated patients’ kidney function declined 4.6 units/year more slowly after injection than before injection — advertised as “about 3× the FDA’s bar.” But that comparison is each patient against their *own past*, not against a control group. Patients entering a trial also get their standard medications tightened up at the same time, and patients selected *because* they were declining fast tend to decline more slowly afterward regardless of treatment. The Phase 2 design cannot separate those effects from the drug. So this is not evidence of a large effect; it is **an absence of measurement**. ([full Phase 2 results](#))
2. **The FDA’s own wording pre-loads exactly that problem.** The agreed accelerated-approval bar is “at least 1.5 mL/min/1.73m²/year improvement” — “*versus sham controls*” and “*in the setting of patients receiving appropriate standard of care therapies*.” The Phase 3 has a sham arm that gets the same medication tightening. So the Phase 3 is measuring a quantity the Phase 2 never estimated.
3. **Therefore our probability of success is just the base rate** for a kidney Phase 3 — call it ~40%, with honest error bars. It is an assumption, not a measurement.
4. **The company must raise money before the answer arrives.** Cash was \$181.6M on 6/30; the quarter burned \$43.3M and research spend is growing 39% year over year. The company’s own 10-Q filed 8/10/2026 says there is “substantial doubt” about its ability to continue as a going concern. Funding through the readout plus a year needs roughly \$180M, which at a normal financing discount means **35–60% more shares**.
5. **Put those together and the stock is priced about right.** At ~\$1.51 and ~440M shares after the raise, using the sell-side’s *already-cut* peak sales estimate of \$900M, today’s price pays for roughly a 42% chance of success. That is essentially our base rate. There is no gap to trade.
6. **The downside is not bounded.** One asset, going concern, one binary date. A miss is a stock around \$0.20–0.40, not \$1.07. A fairly priced bet with an uncapped downside is a pass, not a

hold-your-nose buy.

Net: FLAT. No starter position. No automatic re-entry just because the price falls.

What the market believes

- **The bulls** capitalize the 4.6 number as though it were a controlled treatment effect, and mark the company against a multi-hundred-million to billion-dollar peak-sales outcome.
- **The marginal seller has already moved.** BofA cut its peak sales estimate from \$1.8B to \$900M. Standing price targets of \$9 (Citi), \$8 (UBS) and \$3 (Morgan Stanley, Hold) are July-2025 artifacts sitting against a \$1.51 tape — nobody is trading off them.
- **The tape reads distress, not disbelief in the science:** -57% from the 52-week high, ~16% off the low of \$1.30, with a \$200M at-the-market share-sale program authorized — roughly 44% of the company's market value in potential new supply hanging over it.
- **On 8/21 the stock moved +10.2% intraday with no filing behind it.** We could not explain that move. It is unexplained, not dismissed.

So the market is not asleep. It is pricing a real drug with a real regulatory agreement, discounted hard for money it does not yet have.

Why we might have edge

Honestly stated, and each one is thin:

- **We read the trial design, not the press release.** The distinction between a within-patient before/after comparison and a sham-controlled comparison is the whole thesis, and retail-facing coverage repeats “3× the FDA bar” without it. If the market is systematically over-reading that number, we are correctly on the other side.
- **We did the two halves of the math together.** One of our own benches built a per-share value table on today's share count while separately forecasting 46–50% more shares — and concluded “financing is not the binding constraint.” Multiply the two and financing *is* the binding constraint. That mistake is easy to make and probably common in published notes on this name.
- **We know how the money will actually be raised.** A \$200M need against a ~\$455M company cannot be dribbled into the market quietly; it almost certainly requires an announced deal. That tells us where to watch and roughly when.
- **We can see the insider record.** A sitting director sold his entire common holding on 8/18–8/20, eight days after the going-concern disclosure.

Why it might be priced in

Symmetrically, here is why we probably have no edge at all:

- **The going-concern language is on the public record**, in the company's own filing, since 8/10. Nothing about the funding problem is private information.
- **The dilution math is arithmetic anyone can do.** Cash, burn rate, and the size of the share-sale program are all disclosed.
- **The sell side already cut.** We are agreeing with the analyst who moved, not disagreeing with the consensus. Agreeing with the marginal mark is not edge.
- **The insider sale is timing-explainable.** 8/18 was the first trading window after the quarterly filing — the normal time for insiders to transact. His shares were bought for cash in January 2022, before the company went public via merger, almost certainly around \$10. Selling at \$1.37 is realizing an ~86% loss. That is a person cutting a bad trade, which is only weakly informative about what he thinks the 2027 readout will show.
- **Our probability of success is a guess.** If the true peak sales are nearer the old \$1.8B than the cut \$900M, the dilution-adjusted value at a 40% success rate is \$2.86 — the stock nearly doubles and we were wrong to pass.
- **The price already sits at the base-rate value.** “Fairly priced” is the *definition* of no edge.

What we checked ourselves

Including, plainly, where our own two benches got things wrong.

Claim	How checked	Found	Confirmed?
“4.6 units/year is ~3× the FDA bar and transfers to Phase 3”	Company's own Phase 2 results release; two trade-press readouts of the arm design	The 4.6 is a before/after comparison within the same patients. It cannot separate drug effect from medication tightening at trial entry or from natural regression in fast decliners.	No — refuted as evidence. The number is real; the inference is not.
The FDA bar is a plain ≥1.5 threshold	Verbatim re-pull of the FDA-alignment release, done independently by both benches	Bar is “at least 1.5... versus sham controls... receiving appropriate standard of care therapies. ” Only 37% of Phase 2 patients were on modern SGLT2 inhibitors, so there is a lot of medication headroom to confound the before/after read.	Yes — confirmed, and it cuts against the thesis
OUR BENCH ERRED: brief claimed the low-dose group showed “no slowing at all”	Trade-press readout of Group 2	Group 2 slowed by 1.7 units/year, p=0.085. Not null.	Bench error — corrected by our own red bench
OUR BENCH ERRED: red claimed the dose response is “textbook regression to the mean,” 6× more separation before treatment than after	Blue bench re-read the trial design	Group 2 was not a half-dose arm — patients who worsened got a second injection under protocol. So the small post-treatment gap is partly by design. And a 2.4-unit baseline gap at n≈25 is about 1.4 standard deviations (p≈0.16) — unremarkable.	Red over-claimed. Correct verdict is “uninformative,” not “artifact.” Severity drops from fatal to sizing.
OUR BENCH ERRED: red claimed	Blue bench pulled the Q1-2026 results release	The release states 470 subjects and explains the 1H-2024 amendment to	Red inverted the sign. Finding

Claim	How checked	Found	Confirmed?
the Phase 3 was quietly cut from 685 to 470 patients with no explanation		focus on more advanced kidney disease — three months <i>before</i> the quarter red flagged. Enriching for faster progressors preserves or improves the trial's power at lower headcount.	withdrawn.
OUR BENCH ERRED: red's value table showed the stock clearing +50% at a 40% success rate, concluding "financing is not the binding constraint"	Blue applied red's own dilution forecast to red's own table	At 440M shares the 40% line is \$1.43 against a \$1.51 stock — a 5% <i>discount</i> , not a 38% premium. Financing is the binding constraint.	Red's verdict survived; red's stated reason did not.
OUR BENCH ERRED: brief feared the share count understated enterprise value	Red pulled the 10-Q cover page directly	208.9M Class A + 93.4M Class B = 302.3M, and the Class B shares are already paired 1:1 with exchangeable units. 302.3M is the full economic count.	Red killed its own side's bear point. Correct call. SEC 10-Q cover
Director sold his entire stake	Form 4 and Form 144 pulled from SEC directly	Brian J.G. Pereira, sitting director re-elected through 2029, sold 675,235 trust + 81,929 direct shares on 8/18–8/20 at \$1.345–\$1.50. Shares were bought for cash 1/17/2022. Class A holdings to zero.	Yes, but softened. Table II of the Form 4 is empty, and Table II reports derivative transactions, not standing holdings — so "he owns nothing" is unproven. The proxy ownership table is the right document and we have not read it. Form 4
OUR BENCH ERRED (minor): red reported 82,929 direct shares sold	Blue re-added the three line items	81,929.	Trivial, corrected
Cash, burn, runway	Company 10-Q + quarterly release, 8/10/2026	\$74.9M cash + \$106.7M securities = \$181.6M; Q2 burn \$43.3M; R&D \$36.1M vs \$25.9M a year ago. Runway ends <i>at</i> the readout.	Yes
The company says it is funded "into mid-2027"	Compared the same-day press release to the same-day 10-Q	The press release says "into mid-2027" and contains no going-concern language. The 10-Q, filed the same day, concludes "substantial doubt." The Q1 release carried the same runway wording with no such caveat.	Yes — real disclosure divergence. The company chose the softer venue for the softer message.
OUR BENCH ERRED: red said the plan to "revisit after the raise clears" is unexecutable because an at-the-market program never prints a clearing event	Blue checked the size against the market cap	Red is right about the mechanism, but a \$200M need against a ~\$455M company cannot be absorbed quietly; that mathematically forces an announced deal. We could not confirm this — daily volume data was unavailable.	Partly. Watch both channels, not one.

Claim	How checked	Found	Confirmed?
OUR BENCH ERRED (stale price): red priced the whole case off \$1.3701	Blue's pack was built 78 minutes later at \$1.51	Every market-cap and value figure in red's case is one session-move stale — a +10.2% intraday move with no filing behind it.	Yes. All numbers in this deck use ~\$1.505–1.51.
Trading liquidity and short interest	Attempted broker data pull	Denied — both benches. No daily volume, no short interest, no read on how contested an exit would be.	No — open coverage gap, reported not skipped
Manufacturing plant activity	Considered satellite thermal read on the Winston-Salem facility (36.09981/–80.24405)	Not run. Both benches flagged this as genuinely worth doing — the cost of making an individualized cell therapy is the hidden variable separating the \$900M and \$1.8B sales scenarios.	No — not run
Competitor pipeline against the company's own competition disclosure	Trial-registry sweep	Not run.	No — not run

PROPOSED ENTRY BANDS

Both benches say no entry. There is no buy band today, at any price, on price alone. What follows are reopen conditions, stated with the benches' own arithmetic.

Reference math (blue bench, at ~\$1.51 and ~440M shares after a raise, using BofA's cut \$900M peak):

Chance of success	Value per share	vs. \$1.51
30%	\$1.07	–29%
40% (our base rate)	\$1.43	–5%
50%	\$1.79	+19%
60%	\$2.15	+42%
64%	~\$2.27	+50%

At the old \$1.8B peak, the 40% line is \$2.86. At a bear \$400M peak, the 40% line is \$0.64. A failed readout is worth roughly \$0.20–0.40.

Reopen requires BOTH of the following. Neither alone is sufficient.

- **Trigger A — the money is raised, at a known price.** An announced, sized offering (a 424B5 prospectus supplement, registered direct, or PIPE) that establishes a clearing price. Watch in parallel for the quieter channel: the Class A share count on the cover of each successive 10-Q/10-K. If that count climbs materially with no announced deal, the at-the-market program is doing the work and there will be no announcement to wait for.
- **Trigger B — a real comparison group exists.** Any estimate of rilparencel's effect *against a separate control* — the Phase 2 protocol and statistical plan showing the baseline period was prospectively defined under already-optimized medication with balanced arms, a matched

external-control analysis, or the count of Group 2 patients who crossed over to a second injection. **Absent Trigger B, a cheap post-raise price is still buying an unmeasured coin flip.** Do not skip this one.

If and only if both fire, the indicative bands would be:

- **Starter, minimum size, 0.25% of the book:** at or below **\$0.90** — the price that pays for a 25% chance of success against the cut peak, i.e. a genuine discount to base rate with the financing resolved.
- **Add:** at or below **\$0.64** — the price that pays for a 40% chance even under the bear \$400M peak scenario. This is the “wrong about the drug’s commercial size and still fine” level.
- **No add above \$1.07** under any circumstance, since that is the 30%-chance line and offers no margin.
- **Hard stop on the thesis:** a failed or ambiguous Phase 3 readout. There is no partial credit here and no trimming plan that survives it — the tail is \$0.20–0.40 and we should size as if that is the outcome.

Sizing if reopened: maximum 0.5% of the book, all-in, across both tranches. One asset, one date, going concern, unbounded downside. This does not qualify as a risk we can be fairly paid to own, because “fairly paid” requires the loss to be bounded and here it is not.

Alert-only price levels (no order attached): below **\$1.30** (52-week low breach — reads as financing pressure arriving), below **\$1.07** (30%-chance line — deterioration beyond our base case).

Catalyst dates:

Date	What	Confirmed?
2026-11-09	Next quarterly results	No — derived from a data vendor’s calendar, cadence-consistent with November 2025. No company release names it. ~57 trading days out, so not imminent.
H2 2026 (through 2026-12-31)	Full enrollment of ~470 patients in PROACT 1	Company-stated window, not a day
Mid-2027	Cash exhaustion per company’s own runway language, and the point at which the 10-Q’s substantial-doubt window bites	Company-stated window
~Q2 2027	PROACT 1 interim kidney-function readout — the binary	Company-stated window, not a day
H2 2029	Confirmatory composite readout	Company-stated window

The live hazard is not any of these dates. It is the financing, which can arrive on any morning without a calendar.

ARMED SENSORS

(armed provisionally with this deck; session adjudication ratifies)

- price alert: fires below 0.64 (deck-provisional: 40%-chance value under the bear \$400M peak scenario — candidate add level, same two-trigger precondition)
- filing watch: 424B5,S-3,S-3ASR,8-K until 2027-02-17 (deck-provisional: announced, sized offering establishing a clearing price — reopen trigger A; 8-K Items 1.01 and 3.02 ca)
- dated pack PROK|2026-11-09: PROK: next quarterly results — vendor-derived from prior-year cadence, no company release names it (date confirmed=False)
- dated pack PROK|2026-12-31: PROK: end of company-stated H2-2026 window for full enrollment of ~470 patients in PROACT 1 (date confirmed=False)
- dated pack PROK|2027-06-30: PROK: company-stated mid-2027 cash exhaustion; also the approximate PROACT 1 interim kidney-function readout — the binary event (date confirmed=False)
- dated pack PROK|2029-12-31: PROK: end of company-stated H2-2029 window for the confirmatory composite readout (date confirmed=False)

What remains unverified

Stated as gaps, not as reassurance:

1. **The true chance of success.** Our 40% is a category base rate, not a measurement of this drug. Every valuation number in this deck is downstream of it.
2. **Whether the director actually holds nothing.** The Form 4's derivative section was empty, and that section reports transactions rather than holdings. He may still hold exchangeable units or options. The proxy ownership table settles it; we have not read it.
3. **The crossover count in Phase 2** — how many low-arm patients received a second injection. This number decides whether the trial can say anything about dose at all. Not disclosed in any source we read.
4. **Whether the baseline decline period was prospectively defined,** and whether the two arms were balanced on starting kidney function and medications. Requires the protocol and statistical plan.
5. **Daily trading volume and short interest.** Broker connection denied to both benches. We do not know how contested an exit would be, or whether the at-the-market program could in fact absorb \$200M quietly — which is the load-bearing assumption in blue's argument that an announced deal is coming.
6. **Manufacturing cost and capacity.** Never examined. It sets the gross margin in every sales scenario above.
7. **The competitive field.** No sweep of other kidney-function-slope programs against the company's own competition disclosure was run.
8. **The 2026-11-09 earnings date.** Vendor-derived. No company release confirms it.
9. **The 8/21 price move.** Unexplained.

10. **The peak sales figure itself.** We used \$900M because that is where the analyst who most recently moved put it. We have not built our own commercial model, and the difference between \$900M and \$1.8B is the difference between “pass” and “double.”

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